



TRACK 0 INFRASTRUCTURE · OPENEMR

OpenEMR 8.4 Master Setup Guide

From installation through billing through deposits, aligned to the GFC Care Platform build

Version 4.0 · September 5, 2026

Consolidates and supersedes: *Deploy & Setup Guide v2.3* (July 22, 2026), *Billing & Configuration Guide v3.0* (September 3, 2026), and the OpenEMR track of the *Release Runway* (September 5, 2026).

Aligned to the repo at Session 4.4 merged (PR #29); Sessions 4.5 and 4.3 pending.

Target platform: OpenEMR 8.4.0 on AWS EC2 + RDS, us-east-1.

Test data only until Session 5 signs off

No real patient PHI enters OpenEMR or the app until the clinical HIPAA go-live gate is passed. Every patient created during setup is a named test record (TEST, PatientOne) and is purged before go-live. Passwords and secrets are never written in this guide. Names of secrets only.

Godwins Family Care LLC · Internal reference · Owner: Bianca Ume · EMR maintainer and app team share this document. Where this guide and the app spec disagree, the spec wins; where this guide and an older guide disagree, this guide wins.

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How to use this guide

This is one document in place of three. The Deploy guide told you how to stand the server up. The Billing guide told you how to configure identity, codes, and claims. The Release Runway told you what to do this week. They overlapped, they disagreed in places, and they were all written against OpenEMR 7.0.4. This version is written against 8.4, walks the whole path in order, and carries the app alignment layer so the EMR side and the app side stop drifting.

Reconciliation rules applied

- **v3.0 wins over v2.3** everywhere they disagreed: fee schedule at 125 to 150% of allowable (not Medicare rates), clinicians get Fee Sheet write, POS per facility (never a blanket 12), modifiers configured, commercial payers only with a contract, organizational NPI populated once the 855B lands.
- **The spine is a fresh install on 8.4.** Because the live instance is already on 7.0.4, Phase 6A is the upgrade path from the Runway. Follow Phases 0 to 6 only if you are rebuilding from nothing; follow 6A if you are upgrading what is live.
- **Menu paths carry a marker.** Both source guides were written against 7.0.4. 8.x moved some menus. Any path marked **VERIFY ON 8.4** is the 7.x path; confirm it on the upgraded instance in 6A step 4 and correct it here. If a path does not match, use the search box in OpenEMR's left menu.
- **App alignment is a first-class part.** Part VII maps every EMR configuration value to the app setting that consumes it, records what 8.4 changes for the app, and lists the workarounds Session 4.5 retires.

Markers used

VERIFY ON 8.4 7.x menu path, confirm after the upgrade **APP** the app reads or writes this value **OWNER** Bianca's decision or an outside filing, not an EMR click **DONE** already executed on the live instance **DEFERRED** deliberately later, with a trigger

The release sequence this guide serves

WHEN	OPENEMR (TRACK A, THIS GUIDE)	APP (TRACK B, CLAUDE CODE SESSIONS)
Sat	Snapshot both halves. Upgrade production to 8.4 in place. Verify. Re-register the OAuth client (Phase 6A).	—
Sun–Mon	Write and test the orders + fee-sheet route patch (Phase 6B).	Write the Session 4.5 prompt.
Mon	Configure: ACL, ICD–10 load, security, org, facilities, providers, codes, modifiers, fee schedule, payers (Parts II–III).	Run 4.5. Native writes first, retire workarounds in a second commit.
Mon–Tue	Per-patient setup for the first visit's patient. Acceptance test (Appendix A).	Run 4.3. Patient read, POA gates, case-manager read.
Tue	Set billing NPI and clinician NPIs in the app. Walk one full visit end to end.	Merge, verify, release.

Sunday-night decision point

If the in-place upgrade fails and the rollback in 6A is what you are running on Sunday night, or the route patch looks unstable, stop and fall back: stay on 7.0.4, apply the two one-line bug patches (you have 8.4's exact code for both, so it is a copy, not a debug), and ship with orders and fee-sheet charges entered through OpenEMR's own screens under Bethel's login. That still produces a complete, billable visit on Tuesday. It carries a manual step you remove later.

PART I

Install and deploy on OpenEMR 8.4

Phases 0 to 6 are the deployment record from v2.3, carried forward and pinned to 8.4.0. They are marked **done** because the live instance already exists. Read them if you ever rebuild. Phase 6A is the in-place upgrade you run today. Phase 6B is the patch that keeps write capability for orders and charges.

What this runs on, and what it costs

ITEM	VALUE
Application	OpenEMR, Docker image <code>openemr/openemr</code> , pinned tag. Live today: 7.0.4. Target: 8.4.0.
Host	EC2 t3.small, Ubuntu 24.04, 30 GB encrypted gp3, instance name <code>gfc-openemr</code> , Elastic IP
Database	RDS MySQL 8.0, db.t4g.micro, encrypted, 20 GB gp3 autoscaling to 100 GB, 30-day automated backups, identifier <code>gfc-openemr-db</code>
Address	<code>https://emr.godwinsfamilycarellc.com</code> (Let's Encrypt, issued by the container)
Region	us-east-1. Everything in one region, always.
Run cost	About \$40 to \$55 per month. Budget alarm at \$80.
AWS BAA	Accepted via AWS Artifact DONE
Backups	Daily EBS snapshots (14-day retention) via Lifecycle Manager; RDS automated (30-day)

Not Lightsail, not the retired Marketplace listing

Lightsail is not on AWS's HIPAA-eligible services list. EC2 and RDS are. The AWS Marketplace "OpenEMR Cloud Standard" listing has been retired and the remaining Express listing is not HIPAA-eligible. This hand-built EC2 + RDS pair is the equivalent of the Standard architecture at roughly a sixth of the ECS price.

Phase 0 · Decisions, worksheet, prerequisites

DONE ON THE LIVE INSTANCE

- Worksheet first.** Region us-east-1. EMR subdomain `emr.godwinsfamilycarellc.com`. Three generated passwords in the password manager, named DB-MASTER (RDS master), DB-OPENEMR (OpenEMR's own database user), OE-ADMIN (the OpenEMR admin login). Twenty or more characters, letters and numbers only. Quotes, spaces, `$`, and `#` break the compose file.
- AWS BAA.** Console → AWS Artifact → Agreements → Business Associate Addendum shows accepted.
- DNS access.** Confirm you can log in wherever `godwinsfamilycarellc.com`'s records are managed. One A record is added in Phase 4.

- 4 **No orphans.** EC2 → Instances: nothing running except `gfc-openemr`. Elastic IPs: none unattached. Volumes: none "available". Unattached resources bill every hour.

Passwords that passed through chat get rotated

The DB-MASTER, DB-OPENEMR, and OE-ADMIN values used during the July build were typed into a chat window. Rotate all three before Session 5 go-live (Part VI control C-9). Rotation on RDS is Modify → new master password; OpenEMR's own user and the compose file follow.

Phase 1 · Security groups

DONE · 15 MIN

- 1 `gfc-emr-web`, "OpenEMR web server". Inbound: HTTPS 443 from Anywhere-IPv4; HTTP 80 from Anywhere-IPv4 (certificate check and redirect only); SSH 22 from My IP; SSH 22 from `18.206.107.24/29` (EC2 Instance Connect range for us-east-1, so the browser terminal can reach the box). Outbound default.
- 2 `gfc-emr-db`, "OpenEMR database". Inbound: MYSQL/Aurora 3306 with source `gfc-emr-web`. Nothing else. The database is never exposed to the internet.

If Safari cannot connect after install

The container is usually up. The missing piece is the HTTPS 443 inbound rule on `gfc-emr-web`. This was the July finding.

Phase 2 · The database (RDS)

DONE · 20 MIN + 10 MIN WAIT

- 1 RDS → Create database → Standard create → MySQL 8.0 (latest offered) → template Production → Single DB instance.
- 2 Identifier `gfc-openemr-db`. Master username `dbadmin`. Master password DB-MASTER (self-managed).
- 3 `db.t4g.micro`. Storage 20 GB gp3, autoscaling on, max 100 GB. This is enough: OpenEMR's typed chart is small, and scanned documents live on the EBS volume, not in the database. Revisit at volume.
- 4 Connectivity: do not connect to an EC2 resource; default VPC; public access No; security group `gfc-emr-db` only.
- 5 Additional configuration: **leave "Initial database name" blank**. The installer creates `openemr` itself. Pre-creating it halts the install with "Can't create database 'openemr'; database exists." Backup retention 30 days. Encryption enabled (default KMS key). Deletion protection on.
- 6 When Available, copy the endpoint (`gfc-openemr-db.xxxxxxxx.us-east-1.rds.amazonaws.com`) to the worksheet.

Phase 3 · The web server and a permanent address

DONE · 20 MIN

- 1 EC2 → Launch instance. Name `gfc-openemr`. Ubuntu Server 24.04 LTS x86. t3.small. Key pair `gfc-emr-key` (the `.pem` is break-glass only; the browser terminal does not need it).

- 2 Network: auto-assign public IP on; existing security group `gfc-emr-web`. Storage 30 GB gp3, Advanced → Encrypted yes.
- 3 Elastic IPs → Allocate → Associate with `gfc-openemr`. Copy the address to the worksheet. Without it the address changes on every restart and the domain breaks.

Phase 4 · DNS

DONE · 10 MIN + PROPAGATION

- 1 At the DNS host: add a record, type A, host `emr`, value the Elastic IP, default TTL.
- 2 Verify at dnschecker.org before starting Phase 5. The installer requests the HTTPS certificate during setup and the certificate authority verifies the domain by visiting it. If DNS is not pointing at the server, issuance fails and the install step repeats.

Phase 5 · Install OpenEMR 8.4

FRESH-BUILD PATH · 40 MIN

Not for the live server

Phase 5 is for a machine with nothing on it. On the live `gfc-openemr` box, step 1 reports "already the newest version" and "swapfile is mounted", and step 2 would overwrite the working compose file with placeholders and take the EMR down. If you see those messages, remove the duplicate fstab line with `sudo sed -i '$ d' /etc/fstab` and go to Phase 6A.

Everything here runs in a terminal that opens in a browser tab: EC2 → Instances → `gfc-openemr` → Connect → EC2 Instance Connect. You are typing on the server. Nothing touches your Mac.

- 1 **Prepare the server.** Paste each block, press Enter, wait for the prompt.

```
sudo apt-get update && sudo apt-get -y upgrade
```

```
sudo apt-get -y install docker.io docker-compose-v2
sudo systemctl enable --now docker
```

```
sudo fallocate -l 2G /swapfile && sudo chmod 600 /swapfile
sudo mkswap /swapfile && sudo swapon /swapfile
echo '/swapfile none swap sw 0 0' | sudo tee -a /etc/fstab
```

If the swap block says "Text file busy" or "already mounted", swap is already active. Check with `swapon -show`. If `/etc/fstab` gained a duplicate line, remove the last line with `sudo sed -i '$ d' /etc/fstab`.

- 2 **Write the compose file.** Replace the placeholders with worksheet values. The image tag is the only line that differs from the July build.

```
sudo mkdir -p /opt/openemr && cd /opt/openemr
sudo tee docker-compose.yml > /dev/null <<'EOF'
services:
  openemr:
    image: openemr/openemr:8.4.0
    restart: always
    ports:
      - "80:80"
      - "443:443"
    environment:
      MYSQL_HOST: "<<RDS_ENDPOINT>>"
      MYSQL_ROOT_USER: "dbadmin"
      MYSQL_ROOT_PASS: "<<DB-MASTER>>"
      MYSQL_USER: "openemr"
      MYSQL_PASS: "<<DB-OPENEMR>>"
      MYSQL_DATABASE: "openemr"
      OE_USER: "admin"
      OE_PASS: "<<OE-ADMIN>>"
      DOMAIN: "emr.godwinsfamilycarellc.com"
      EMAIL: "info@godwinsfamilycarellc.com"
    volumes:
      - sitevolume:/var/www/localhost/htdocs/openemr/sites
      - ssl:/etc/ssl
      - letsencrypt:/etc/letsencrypt
volumes:
  sitevolume:
  ssl:
  letsencrypt:
EOF
```

- 3 **Launch and watch.**

```
sudo docker compose up -d
sudo docker compose logs -f
```

First run takes 5 to 10 minutes: pulls the image, builds the tables on RDS, creates the `openemr` database user, requests the certificate. Wait for `setup-complete` / Apache started, then Ctrl+C (the server keeps running).

- 4 **Recovery notes.** Certificate failure means DNS was not ready: fix Phase 4, then `sudo docker compose down && sudo docker compose up -d`. "Can't create database 'openemr'; database exists" means an initial database name was set in Phase 2: `sudo docker compose down`, then drop the empty database with a throwaway client, `sudo docker run --rm mysql:8.0 mysql -h <RDS endpoint> -u dbadmin -p'<DB-MASTER>' -e "DROP DATABASE IF EXISTS openemr;"`, then up again. Reaching that error proves the credentials and firewall are right.

- 5 **First login.** <https://emr.godwinsfamilycarellc.com> with a valid padlock. Username `admin`, password `OE-ADMIN`. Do nothing else yet. Configuration is Part II, in order.

Why pin the tag

A pinned tag means the server never surprise-upgrades itself. The monthly routine (Part VI) pulls the latest patch build of the pinned line. Moving to a new major or minor is a deliberate event behind a snapshot, exactly as Phase 6A does.

Phase 6 · Verify, back up, cost alarm

DONE · 30 MIN

- 1 RDS → `gfc-openemr-db` → Configuration: Encryption enabled. Maintenance & backups: automated, 30-day retention.
- 2 EC2 → Elastic Block Store → Lifecycle Manager → EBS snapshot policy targeting volumes tagged Name = `gfc-openemr`, daily 09:00 UTC, retain 14.
- 3 Billing → Budgets → Monthly cost budget, \$80, to your email.
- 4 Baseline snapshot of the volume, description "post-install clean baseline". Your known-good restore point before any configuration.
- 5 MFA on the AWS account itself (IAM → Security credentials → Assign MFA device). The console can destroy or expose everything below it.

Phase 6A · Upgrade the live 7.0.4 instance to 8.4, in place

TODAY · ABOUT 1 HOUR

Verified against 8.4 source: it adds `POST /api/prescription`, and it fixes both server bugs the app has been working around, the document-endpoint 500 (missing bind array at `DocumentService.php:92`) and the vitals 500 (null `authUserId`). The two local patches planned for 7.0.4 are no longer needed.

Decision (Sept 5): no scratch rehearsal. The upgrade runs directly on the live instance. That is acceptable because the instance holds test data only, so the worst case is downtime, not data loss. What makes it safe is step 1. The two snapshots are the rehearsal's replacement: if the upgrade fails, you restore and you are back on 7.0.4 in about 20 minutes. Do not start step 3 until both snapshots read Completed.

- 1 **Snapshot both halves.** In the browser terminal, stop the container so the disk is quiet: `cd /opt/openemr && sudo docker compose stop`. Then in the console: RDS → Databases → `gfc-openemr-db` → Actions → Take snapshot, name `pre-84-upgrade`. EC2 → Volumes → the `gfc-openemr` volume → Actions → Create snapshot, description "pre-8.4 upgrade". Wait for both to read **Completed** (RDS 5 to 10 minutes, EBS a few minutes). The EMR is offline during this window; that is fine.
- 2 **Change one line.** Do not rewrite the compose file. The existing file already holds the real RDS endpoint and passwords. Swap only the image tag and confirm:

```
cd /opt/openemr && sudo sed -i 's#openemr/openemr:7.0.4#openemr/openemr:8.4.0#' docker-compose.yml && grep image docker-compose.yml
```

The output must read `image: openemr/openemr:8.4.0`. If it still says 7.0.4, the tag in the file is written differently; open it with `sudo nano docker-compose.yml`, change the tag by hand, Ctrl+O, Enter, Ctrl+X.

3 Pull and start. Then leave it alone.

```
sudo docker compose pull && sudo docker compose up -d
sudo docker compose logs -f
```

Six schema upgrades run in sequence against RDS. Do not interrupt, do not close the browser tab, do not run `down`. Expect 10 to 20 minutes on a `t4g.micro`. You are done when the log settles on Apache running (lines like `httpd` started or setup complete) and stops scrolling. Ctrl+C exits the log view; the container keeps running.

- 4 **Verify production.** Browse to <https://emr.godwinsfamilycarellc.com> (the certificate lives on the `ssl/letsencrypt` volumes and survives the image change). Admin login. Administration → About shows 8.4. Open TEST PatientOne's chart. Open an encounter's Fee Sheet. Fees → Billing Manager. Administration → Coding → Codes shows the entered CPT rows. Facilities and Users intact. Administration → Config → Connectors still shows both APIs enabled and the site address unchanged. **Write down every menu path that moved.** This step is what clears the **VERIFY ON 8.4** markers in the rest of this guide.

- 5 **Re-register the app's OAuth client** with the 8.4 scope set (Phase 9.3). Scopes bind at registration and cannot be widened, so a new scope set always means a new client. Enable it, take a token with the password grant, swap the two env values in the app, and run the 8.4 probes in Part VII.4: `POST .../vital` returns a row, `GET/POST .../document` no longer 500s, `POST /api/prescription` exists. Record the results in the compatibility matrix.

- 6 **Post-upgrade baseline.** Audit log still recording (Administration → System → Logs shows your logins from step 4). Take a snapshot of the upgraded volume, description "post-8.4 clean baseline". Delete nothing; the `pre-84-upgrade` snapshots stay until the acceptance test passes.

Rollback, if step 3 or 4 fails

Read the log error first; a pull failure or a bad tag is a retry, not a rollback. If the schema upgrade itself errors or the site will not load: (1) `sudo docker compose down`. (2) RDS → Snapshots → `pre-84-upgrade` → Actions → Restore snapshot → identifier `gfc-openemr-db-restore`, same class `db.t4g.micro`, same VPC, public access No, security group `gfc-emr-db`. Wait for Available and copy its endpoint. (3) Edit the compose file: `MYSQL_HOST` to the restored endpoint, tag back to `7.0.4`. (4) `sudo docker compose up -d`. You are on 7.0.4 with the pre-upgrade database. The original RDS instance can then be deleted once you have confirmed the restore works, or kept for a second attempt. Then apply the two one-line 7.0.4 bug patches from 8.4's code and continue on the fallback path: orders and fee-sheet charges entered through OpenEMR's own screens under Bethel's login until the upgrade is retried.

Phase 6B · The orders and fee-sheet route patch

AFTER 6A · EMR MAINTAINER

This is the finding that keeps write capability. 8.4 still ships no API route for order writes or fee-sheet charges, but the business logic already exists and is what OpenEMR's own screens call.

`BillingUtilities::addBilling()` is the exact function the Fee Sheet uses to insert a charge.

`ProcedureService` already carries `addDiagnosis`, `updateOrderCode`, and `deleteOrderCode`. The patch adds controller methods and route lines over logic that is already tested in production use. It does not write billing logic from scratch.

Scope, in order of value

1. **POST a fee-sheet charge for an encounter**, wrapping `BillingUtilities::addBilling()`. Carries CPT/HCPCS code, modifiers, units, ICD-10 pointers (the linked encounter diagnoses), rendering provider, billing facility. This is what makes the app's sign-and-close land in Billing Manager.
2. **POST a procedure order** plus its order-code rows over `ProcedureService`. Create is the only missing path.
3. **Optional: a code-search route** over the loaded ICD-10 and CPT tables. Removes the last app-side coding workaround (the app currently seeds the picker only from the patient's problem list, the clinician's own history, and the admin favorites).

Rules for the patch

- Keep it narrow. New routes only. No changes to existing controllers beyond registration lines.
- Reuse existing scopes where the semantics fit (fee-sheet charge under `user/encounter.write`, orders under a `user/procedure.write` if 8.4 defines it, else `user/encounter.write`). Document which scope guards each route so Phase 9.3's client carries it.
- Record it: file paths, diff, date, OpenEMR version. Store the diff in the repo under `docs/openemr-patches/` and in the EMR maintainer's notes. **A later image pull silently reverts it**, so the monthly routine (Part VI) re-applies and re-tests the patch after every `compose pull`.
- Prefer a build that survives pulls: a bind-mounted override of the two touched files, or a small derived image (FROM `openemr/openemr:8.4.0` plus `COPY` of the patched files) tagged `gfc/openemr:8.4.0-p1`. The derived image is cleaner and the compose tag change is one line.
- If it is clean, upstream it. OpenEMR is open source and these are gaps others hit.

Acceptance for the patch

With the app's token: a charge POSTed to a TEST encounter appears in Fees → Billing Manager with the right code, modifier, provider, and diagnosis pointers; an order POSTed appears under the patient's procedures with its order codes; a non-clinical token gets 401/403. Screenshots and the raw responses go into the defects record as the closing entry for Gap 1.

PART II

Configure identity, practice, access, and the API

Configure once, on the version you will actually run. That is why this comes after the upgrade. Phase 7 is the set of decisions and filings that live outside OpenEMR but decide what you type into it. Phases 8 to 10 are the clicks.

Phase 7 · Identity decisions outside OpenEMR

OWNER · NOT EMR CLICKS

7.1 Design principle: archetypes, not people

Every provider record, facility record, and permission set in OpenEMR is built from a numbered archetype. Named individuals and named facilities are instances. When you add a clinician, pick an archetype, copy its configuration, fill in that person's identifiers. Same for a facility. If a new person or place does not fit an existing archetype, define a new one here first, then build it. Never configure a one-off.

The claim reconciliation rule this protects

For every billed line, GFC must show that enrollment status, rendering provider, certification and scope, EHR signature, service location and POS, eligibility, diagnosis support, and claim data all describe the same real-world service.

7.2 Entity, NPIs, and enrollment OWNER

Entity decision. All service lines stay under GFC LLC. Medicare filings go under that EIN. The split into separate entities is deferred, not cancelled, with a written trigger to be set. Mitigations while combined: separate books and bank accounts by line, separate insurance policies, documented cost allocation.

LINE	WHAT IT IS	REGULATION	PAYMENT
Private home care	Non-medical personal care, ADLs, companionship	GA DCH Private Home Care Provider license	Private pay. Never touches OpenEMR billing
In-home primary care	A medical practice seeing patients where they live	Clinician licensure	Medicare Part B
In-home psych	Behavioral health in the residence	Clinician licensure and certification	Medicare Part B

None of these is Medicare home health. GFC is not a certified home health agency and should stop signalling it.

NPI	LINE	TAXONOMY	USE
1710678339 (existing Type 2)	Home care	Correct to 253Z00000X In Home Supportive Care or 251J00000X, matched to the DCH license. Currently tagged 251E00000X Home Health, which fits neither line.	Private pay. No Medicare claims

NPI	LINE	TAXONOMY	USE
New Type 2	Clinical practice	193200000X Multi-Specialty Group	Billing entity on every Part B claim once the 855B is approved

Address conflict to resolve first. NPPES has GFC at 4945 Baker Plantation Way, Acworth. The July facility record used 4300 Paces Ferry Road SE, Suite 500, Atlanta. Pick one, then make NPPES, PECOS, the 855B, the IRS record, and OpenEMR agree. Address mismatch is the most common cause of a stalled enrollment.

Enrollment. Reassignment only works if the organization holds an active Part B group enrollment; an NPI is an identifier, not an enrollment. File the CMS-855B (revision 04/29/2026, the only version accepted since 08/03/2026). Reassignment in Section 4H, practitioner signature in 15E. No application fee (a nurse practitioner organization is carved out of the institutional-provider definition at 42 CFR 424.502). Bianca appears in ownership and managing control, never as a practitioner. Each clinician's reassignment is filed separately through PECOS or the current 855; the CMS-855R is retired. An outside primary employer stays primary; GFC is an additional reassignment. Check any clinician's outside employment agreement for exclusivity clauses before filing anything naming them.

Retrospective billing. Billing privileges start at the later of the filing date of an approved application or the date services began at the location, with up to 30 days of retrospective billing where all requirements were met. Operating instruction: document every visit completely, and hold any claim whose rendering provider's enrollment or reassignment is unresolved for that date of service. Timely filing is 12 months for Medicare. Holding a claim costs nothing.

7.3 Provider archetypes and the current roster

ID	ARCHETYPE	TAXONOMY	OPENEMR ACL	RENDERS?	MAY BILL
A1	Primary care NP	363LF0000X	Clinical	Yes	Home/residence E/M, AWW, ACP, TCM, CCM
A2	Dual-certified NP, primary + psych	363LF0000X and 363LP0808X	Clinical	Yes	A1 set plus the psych set
A3	Specialty NP	Per certification	Clinical	Yes	Scope-limited subset, defined at onboarding
A4	Physician (MD/DO)	Per specialty	Clinical	Yes	Full scope; the only Schedule II prescriber
A5	1099 contract examiner (IME/C&P)	Per credential	Clinical, restricted	Yes	IME work only, not Medicare E/M
A6	Non-rendering owner/administrator	None	Administrators	No	Nothing. Ownership and managing control
A7	Billing / coder	None	Accounting	No	Nothing. Charge review and claim work
A8	Front office / scheduling	None	Front Office	No	Nothing. Scheduling and demographics

ID	ARCHETYPE	TAXONOMY	OPENEMR ACL	RENDERS?	MAY BILL
A9	Clinical social worker (LCSW)	1041C0700X	Clinical, behavioral	Yes	Psychotherapy and assessment. No E/M
A10	Master's social worker (LMSW)	104100000X	Clinical, restricted	Not independently	Nothing directly. CoCM care manager under an NP

PERSON	ARCHETYPE	TYPE 1 NPI	NOTES
Bethel Godwins	A1	1902310568	GA license RM186487. Primary employer is an East Point practice; GFC is an additional reassignment
Thanmayie Bethi	A2	1962038307	GA license 246736. NPPES lists only the FNP taxonomy. Psych billing blocked until 363LP0808X is added
Bianca Ume	A6	None	Owner and managing director. On the 855B in ownership and managing control
Collaborating physician	A4	TBD	Required for both NPs' protocol agreements. Also the Schedule II referral path
Billor	A7	—	ACL group configured and empty until hired

Onboarding checklist per rendering clinician (A1–A5, A9)

- ☐ State license verified at the issuing board, dated screenshot retained
- ☐ National certification(s) verified with the issuing body, certificate copy retained
- ☐ Type 1 NPI confirmed in NPPES, every taxonomy they will bill under listed, primary designated deliberately; credential field and practice address accurate
- ☐ CMS–855I filed with reassignment to GFC in the same application
- ☐ OIG LEIE and SAM.gov exclusion screening cleared (hard gate, Part VI C–1)
- ☐ Malpractice coverage confirmed for every scope they will practice
- ☐ DEA registration and Georgia PDMP enrollment, if prescribing
- ☐ Nurse protocol agreement executed and filed with GCMB, if APRN
- ☐ Distinct OpenEMR user: Provider and Authorized flags, individual NPI, all taxonomies, own electronic signature, MFA enrolled (Phase 8.4)
- ☐ App user record: `hasClinicalAccess`, `licenseLevel`, `npi` (10 digits), `openEmrProviderId` set to the numeric OpenEMR provider id APP
- ☐ Credential expiration dates entered in the tracker; code set they are permitted to bill recorded against their user record

No shared logins, ever. One human, one account. The login drives the note signature, which must match the claim.

Social work scope (A9 and A10)

An LCSW (A9) enrolls independently, is paid at 75% of the physician fee schedule, cannot bill E/M under any circumstance, and is unaffected by setting exclusions for GFC's book (personal care homes, assisted living, group homes, private residences are all fine). An LMSW (A10) is not a Medicare-enrollable provider type and

incident-to is not a workaround in a private residence. Three legitimate uses for an LMSW: behavioral health care manager under Collaborative Care (the NP bills 99492–99494 and 99484), private-pay care coordination, and accruing supervised hours toward LCSW. Before staffing the CoCM psychiatric consultant role with an NP rather than a psychiatrist, get written confirmation from Palmetto, because Georgia's Schedule II bar makes "qualified to prescribe the full range of medications" unsettled for an APRN.

Who may bill what (source for the certification-to-code check)

SERVICE	A1 NP	A2 DUAL	A4 MD	A9 LCSW	A10 LMSW
Home/residence E/M 99341–99350	Yes	Yes	Yes	No	No
AWV G0438 / G0439	Yes	Yes	Yes	No	No
ACP 99497	Yes	Yes	Yes	No	No
Psych diagnostic eval 90791	No	Yes	Yes	Yes	No
90792 with medical services	No	Yes	Yes	No	No
Psychotherapy 90832 / 90834 / 90837	No	Yes	Yes	Yes	No
Psychotherapy add-on to E/M (+90833 etc.)	No	Yes	Yes	No	No
Family psychotherapy 90846 / 90847	No	Yes	Yes	Yes	No
CCM 99490 / 99439 / 99487 / 99489	Billing practitioner	Billing practitioner	Billing practitioner	Clinical staff time	Clinical staff time
CCM practitioner-only 99491 / 99437	Yes	Yes	Yes	No	No
CoCM 99492–99494, BHI 99484	Billing practitioner	Billing practitioner	Billing practitioner	Care manager	Care manager
Schedule II prescribing	No	No	Yes	No	No
Payment rate	85%	85%	100%	75%	n/a

7.4 Facility archetypes and place of service

POS describes where the service physically happened. It is not a preference and not a facility-wide default. OpenEMR carries one facility record per physical site, each with its own POS, and the encounter can override.

ID	SETTING	POS	E/M FAMILY
F1	Private residence, patient's own home	12	99341–99350
F2	Assisted living community	13	99341–99350
F3	Group home	14	99341–99350

ID	SETTING	POS	E/M FAMILY
F4	Custodial care facility	33	99341–99350
F5	Skilled nursing facility	31	99304–99310, not the home family
F6	Nursing facility, non-skilled	32	99304–99310
F7	Telehealth, patient at home	10	Per service, plus modifier
F8	Telehealth, patient elsewhere	02	Per service, plus modifier

The Georgia wrinkle. Georgia licenses personal care homes and assisted living communities as separate categories, and neither label maps one-to-one onto a CMS POS. POS follows the setting's actual characteristics. POS 13 is a congregate residential facility with self-contained units, resident needs assessment, and 24/7 on-site support. POS 14 is a residence with shared living areas where residents receive supervision plus custodial and minimal services.

Method for every new facility: obtain the DCH license type, number, and bed count; record the operating model (unit structure, staffing coverage, services delivered or arranged); map to F1–F8; if borderline, request written guidance from Palmetto GBA and file it with the facility record; build the OpenEMR record with that POS; confirm no other entity bills an overlapping professional or facility service for those residents. Do not default to POS 12 because the code family is called "home or residence." That was the July error.

FACILITY	ARCHETYPE	POS	STATUS
Hickory Log Personal Care Homes	F2 or F3, pending	TBD	Blocked on DCH license verification
Private-residence clients (Ellijay and others)	F1	12	Active
Telehealth, behavioral health	F7	10	Pending psych go-live

Per-facility record requirements: legal name, street address, unit, phone, administrator contact; correct POS; the facility's policy on outside practitioners, medication orders, resident consent, records access, emergency escalation; BAA or equivalent where GFC handles the facility's PHI; marked as a service location, with GFC remaining the billing location.

7.5 Georgia scope and nurse protocol agreements

Applies to every APRN archetype. Georgia updated these rules effective 05/25/2026. A delegating physician may cover up to the combined equivalent of eight APRNs or PAs at one time. Agreements are filed with GCMB within 30 days of execution, amendments on the same timeline; an incomplete application goes invalid after three months. On termination, the physician notifies the Board within 10 working days and the APRN within 7 days. Chart review by the delegating physician: 100% of records involving controlled-substance prescriptions at least quarterly, 100% of records with adverse outcomes within 30 days of discovery, 10% of all remaining records annually. That review is real recurring work. Price it into the collaborating physician agreement.

Schedule II. Georgia APRNs remain barred from Schedule I and II, with one narrow emergency exception (hydrocodone or oxycodone, 5-day supply, adult patient, explicit protocol authority, one year post-licensure, biennial CE, DEA updated). Stimulants are Schedule II, so no APRN archetype prescribes Adderall, Ritalin, Vyvanse, or equivalents in Georgia. Antidepressants, most antipsychotics, and mood stabilizers are

unscheduled. Benzodiazepines are Schedule IV and available under protocol. A written Schedule II referral pathway (which A4 physician or outside prescriber, and how the handoff is documented) is required before psych go-live.

Phase 8 · Identity and practice build in OpenEMR

MONDAY · 3 HRS

8.1 Secure the admin account and global security

- 1 Logged in as `admin`: username menu → MFA Management → add TOTP. Every user created below enrolls MFA from their own login the same day. Admin first.
- 2 Administration → Config **VERIFY ON 8.4**, Security tab: idle session timeout 900 seconds; password expiration 90 days; minimum length 12; strong passwords required; lockout after 5 failed logins.
- 3 Logging tab: audit logging on, every patient-record subcategory enabled (views, updates, disclosures). No truncation of PHI-access events. Billing operations must audit-log.
- 4 Appearance tab: application title "Godwins Family Care." **Click Save on each tab.** Settings do not persist tab-to-tab.

8.2 Organization record

Administration → Clinic → Facilities **VERIFY ON 8.4**, edit the default facility. This is the billing entity that populates every claim, superbill, and printed document.

Name	Godwins Family Care LLC. Never "Godwins" alone
Address	The one resolved in 7.2. Same string as NPES, PECOS, 855B, IRS
Phone	(404) 913-6705
Tax ID	GFC LLC's EIN, type EIN
Facility (organizational) NPI	Blank until the 855B is approved. Then the new clinical Type 2 NPI. One field, and it redirects payment from an individual to the business. Until then claims bill under Bethel's individual NPI as rendering provider, held as a configuration value on both sides (Phase 8.6 in OpenEMR, <code>billing_npi_used</code> in the app)
Flags	Billing Location, Accepts Assignment, Service Location all checked

8.3 Facility records

One record per physical site from 7.4, each with its own POS, each marked Service Location with GFC as the billing location. Build the F1 private-residence record now (POS 12). Build the Hickory Log record the day its DCH license type is confirmed, with the POS that decides (13 or 14). Build the F7 telehealth record (POS 10) before psych go-live.

The app's encounter defaults point at one of these APP

The app creates every encounter with `facility_id`, `billing_facility`, and `pos_code` from its environment (`OPENEMR_FACILITY_ID`, currently 3; `OPENEMR_POS_CODE`, currently 12) and `pc_catid` from `OPENEMR_ENCOUNTER_CATEGORY`. After you build the facility records, read each record's numeric id from the Facilities list and record it in the values worksheet (Appendix B). Until Session 4.5 adds a per-visit facility picker, the env default must be the F1 record, and any Hickory Log encounter created from the app needs its facility and POS corrected in OpenEMR before it is coded. Put that on the Session 4.5 prompt: facility selection per visit, sourced from OpenEMR's facility list, POS inherited from the facility record.

8.4 Users and provider records

Administration → Users VERIFY ON 8.4. Build from the archetype table. Every rendering clinician: Provider and Authorized checked, individual NPI, all applicable taxonomies, state license, DEA where relevant, MFA.

- Bethel (A1):** username `bgodwins`; Provider and Authorized; NPI 1902310568; taxonomy 363LF0000X; GA license RM186487; DEA when prescribing controlled substances. ACL: Clinical (8.5).
- Thanmayie (A2):** own user; Provider and Authorized; NPI 1962038307; taxonomies 363LF0000X and 363LP0808X (psych billing stays blocked until NPPEs matches); GA license 246736. ACL: Clinical.
- Bianca (A6):** username `bume`; Provider unchecked; ACL: Administrators. Full visibility including billing, no clinical authorship.
- gfc-app-api (the app's service account)** APP DONE: a dedicated user the app authenticates as during the dev window (Phase 9.4). Not a person. Provider unchecked. Its ACL group must cover every clinical section the app writes, plus `sensitivities` (8.6). Retired at Session 5 when per-user `authorization_code` auth lands.
- IME contractors (A5):** create each 1099 FNP's user the day her readiness gate clears, never in advance. An unused provider login is audit surface.
- Record each provider's numeric id** (visible in the Users list URL or the provider dropdown). The app's scheduling module needs it as `openEmrProviderId` on the clinician's app user record, and the app's encounter default `OPENEMR_PROVIDER_ID` should point at Bethel's id, not the installer's id 1. APP

8.5 Access control, corrected

Administration → ACL for group permissions; Administration → Users → [user] → Access Control for assignments VERIFY ON 8.4. The July guide denied clinicians fee permissions, then had a clinician complete a Fee Sheet in its own acceptance test. Those contradict, and the restrictive reading breaks same-day charge capture. Resolution: **clinicians get Fee Sheet write, not financial visibility**.

ARCHETYPE	ACL GROUP	FEE SHEET	PAYMENTS / AR / ADJUSTMENTS	REPORTS
A1–A5, A9, A10	Clinical (Physicians/Clinicians)	Write	No	Own encounters only
A6	Administrators	Write	Yes	All
A7	Accounting	Write	Yes	Financial

ARCHETYPE	ACL GROUP	FEE SHEET	PAYMENTS / AR / ADJUSTMENTS	REPORTS
A8	Front Office	No	No	Scheduling only
gfc-app-api	Clinical + sensitivities + org-level read (Practitioner, Organization, Coverage)	Write (via the 6B route)	No	n/a

Clients, families, and caregivers never log in to OpenEMR. The native patient portal stays disabled (Phase 10.5). The GFC app is the only patient-facing surface. Its own role model (client, family, POA, case manager, clinician, admin, super admin) is enforced in the app, not mirrored as OpenEMR users.

8.6 Unblock the app APP

Two server-side items found during the Session 4 builds. Both are configuration, not code, and both are why the app has been carrying workarounds.

- 1 **Add **sensitivities** to the **gfc-app-api** ACL group.** The encounter PUT returns 500 because `EncounterService::updateEncounter` answers "You are not authorized to see this encounter" on the sensitivities check and the controller crashes on the string. While the same group is open, grant org-level read so `/fhir/Practitioner`, `/fhir/Organization`, and `/fhir/Coverage` stop returning 403 ("Organization policy does not have permit access resource"); the app's provider list is running in a degraded fallback until they do.
- 2 **Load the ICD-10-CM code set.** Administration → Other → External Data Loads VERIFY ON 8.4, ICD-10, load the current CMS release (reissued each October 1). This is why diagnosis codes vanish on read-back: FHIR Condition returns the problem with text only and no coding when the code table is empty. After the load, re-read TEST PatientOne's problem list through `GET /fhir/Condition` and confirm `coding` is present. Add the reload to the annual calendar (Part VI).
- 3 **Enable ICD-10 for Fee Sheet diagnosis selection** (Administration → Config → Features or Codes VERIFY ON 8.4) so the Fee Sheet's diagnosis picker searches the loaded set.

Phase 9 · APIs, the OAuth2 client for 8.4, the API user, secrets

AFTER 6A · 1.5 HRS

9.1 Enable the APIs DONE ON 7.0.4, RE-CHECK AFTER UPGRADE

- 1 Administration → Config → Connectors VERIFY ON 8.4. Enable OpenEMR Standard REST API (`/api/`) and OpenEMR Standard FHIR REST API (`/fhir/`). Leave the Patient Portal REST API off; the app brokers patient reads server-side.
- 2 Site Address: `https://emr.godwinsfamilycarellc.com` exactly, no trailing slash. A mismatch here is the first cause of token failures.
- 3 OAuth2 App Manual Approval: manual. Nothing self-approves.
- 4 Enable OAuth2 Password Grant: on, for the dev window only (9.4). Turn it off at Session 5 when the app moves to `authorization_code` + `refresh_token`.

- 5 Save. Confirm with `GET https://emr.godwinsfamilycarellc.com/apis/default/fhir/metadata` returning a `CapabilityStatement`. "API is disabled" means step 1 did not save.

9.2 How the app authenticates today, and what changes at Session 5 APP

The app holds one confidential client and one API user. It requests a token with the OAuth2 password grant (`grant_type=password`, `user_role=users`) as `gfc-app-api`, keeps the token server-side, refreshes it with `refresh_token`, and never exposes it to the browser. Every EMR call is written to the app's activity log with the acting app user, role, patient, resource, and action. OpenEMR itself attributes every write to `gfc-app-api`, so the app stamps the acting clinician's name and NPI into each note header, encounter reason, and billing note (the attribution interim from the Clinical Completeness Spec §4). Session 5 replaces this with per-user `authorization_code` auth so OpenEMR's own signature matches the claim; at that point the password grant global is disabled and the API user is retired.

9.3 Register the 8.4 client

Scopes bind at registration and cannot be widened, so a new scope set always means a new client. This is the third registration (4.1 clinical set, 4.2 added appointments, 8.4 adds the items below). Run it against scratch in 6A step 4 and again against production in 6A step 5. Registration is a single-line curl; a pasted multi-line JSON body with line wraps returns "Could not decode request body." FHIR scopes use the resource's PascalCase name (`CarePlan`, `MedicationRequest`); lowercase forms return `invalid_scope`.

The scope set is the app's current `config.js OPENEMR.SCOPES` list plus the 8.4 additions. Current list (41 scopes): `openid offline_access api:oemr api:fhir`; FHIR reads `user/Patient.read user/Patient.write user/Encounter.read user/Condition.read user/AllergyIntolerance.read user/Medication.read user/MedicationRequest.read user/CarePlan.read user/DocumentReference.read user/Binary.read user/Observation.read user/Practitioner.read user/Organization.read user/Coverage.read user/Goal.read user/Procedure.read user/DiagnosticReport.read user/Appointment.read`; standard-API `user/patient.read user/patient.write user/encounter.read user/encounter.write user/vital.read user/vital.write user/medical_problem.read user/medical_problem.write user/medication.read user/medication.write user/allergy.read user/allergy.write user/soap_note.read user/soap_note.write user/document.read user/document.write user/facility.read user/practitioner.read user/insurance.read user/appointment.read user/appointment.write`.

ADD FOR 8.4	WHY
<code>user/prescription.read</code> <code>user/prescription.write</code>	Native prescription record via <code>POST /api/prescription</code> (Session 4.5). Confirm the exact write scope string from 8.4's route table; if only <code>.read</code> exists the POST is guarded elsewhere
<code>user/procedure.read</code> (and <code>.write</code> if defined)	Orders read-back, and the 6B order route
<code>user/list.read</code>	List options (appointment categories, statuses)
<code>user/ValueSet.read</code>	FHIR ValueSet, which serves <code>list_options</code>
<code>user/drug.read</code>	Drug lookups behind the prescription form
Whatever scope 6B declares for the fee-sheet route	Charge write from sign-and-close

- 1 From the browser terminal, one line, with the full scope string in place of the ellipsis:

```
curl -sS -X POST https://emr.godwinsfamilycarellc.com/oauth2/default/registration -H "Content-Type: application/json" -d '{"application_type":"private","client_name":"GFC Care Platform (server) v3 8.4","token_endpoint_auth_method":"client_secret_post","redirect_uris":["https://app.godwinsfamilycarellc.com/oauth/callback"],"scope":"openid offline_access api:oemr api:fhir user/Patient.read ... user/appointment.write user/prescription.read user/prescription.write user/procedure.read user/list.read user/ValueSet.read user/drug.read"}'
```

The response carries `client_id` and `client_secret`. They are shown once.

- 2 Store both in AWS Secrets Manager under `gfc/openemr/oauth-client` (Secrets Manager → Store a new secret → Other type → key/value pairs `client_id`, `client_secret`, plus `api_username` and `api_password` for the dev-window user). Never in the repo. `.env.example` stays blank.
- 3 Enable the client: Administration → System → API Clients **VERIFY ON 8.4** → the new registration → Enable. Then disable the previous client ("GFC Care Platform (server) v2", id beginning `dLHHtxPDc1gS`) and delete the never-enabled duplicate (id beginning `SQNsxClgNw`).
- 4 Swap the app's `OPENEMR_CLIENT_ID` and `OPENEMR_CLIENT_SECRET` in the deployed environment's secrets. Restart. The workspace sync indicator reads the granted scope count from the token; it should show the new total. **APP**

9.4 The API user (dev window)

- 1 Administration → Users: `gfc-app-api`, a strong generated password held in Secrets Manager, ACL per 8.5 (Clinical plus `sensitivities` plus org-level read). Provider unchecked. MFA is not applicable to a password-grant service user; compensate with the ACL scope and the Session 5 retirement date.
- 2 Token test, one line:

```
curl -sS -X POST https://emr.godwinsfamilycarellc.com/oauth2/default/token --data-urlencode "grant_type=password" --data-urlencode "client_id=<CLIENT_ID>" --data-urlencode "client_secret=<CLIENT_SECRET>" --data-urlencode "user_role=users" --data-urlencode "username=gfc-app-api" --data-urlencode "password=<API PASSWORD>" --data-urlencode "scope=openid offline_access api:oemr api:fhir user/Patient.read"
```

A JSON body with `access_token` means the client is enabled, the password grant is on, and the site address matches.

9.5 Smoke test and the hand-off record

- 1 With the token: `GET /apis/default/fhir/Patient` returns a Bundle containing TEST PatientOne.
- 2 Save `/apis/default/fhir/metadata` (the CapabilityStatement) as a file in the repo under `docs/openemr/capability-8.4.json`, and note the Swagger URL `https://emr.godwinsfamilycarellc.com/swagger`. Both are handed to the Session 4.5 build and to the reviewing engineer.
- 3 Walk Part VII.4 row by row against the live 8.4 server with TEST PatientOne and sign the Verified column. That matrix is the contract Session 4.5 codes against.

Background jobs later DEFERRED

System-to-system access (the nightly appointment–confirmation trigger from Session 4.2b, bulk exports) needs a second client with the client_credentials grant, system/* scopes, private_key_jwt and a registered JWKS (RS384). Do not register it until a background job actually exists and the HIPAA go-live gate is live.

Phase 10 · Codes, modifiers, fee schedule, calendar, cleanup

MONDAY · 2.5 HRS

10.1 Codes by archetype

OpenEMR ships with no CPT codes (AMA copyright). Enter only what GFC bills, at Administration → Coding → Codes **VERIFY ON 8.4** → Add New, type CPT4 (HCPCS for G-codes). ICD-10 comes from the 8.6 load, never hand-entered. The app keeps no code list of its own; its picker is fed by OpenEMR's problem list, the clinician's own history, and an admin-maintained favorites list that must mirror this fee schedule exactly. If a code is missing, it is added here, not in app configuration. **APP**

SET	CODES
A1 and A2, primary care	Home/residence E/M new: 99341, 99342, 99344, 99345 (99343 was deleted in 2023; do not enter it). Established: 99347, 99348, 99349, 99350. G0438 initial AWV, G0439 subsequent AWV. 99497 advance care planning. 99495, 99496 transitional care management. Nursing facility 99304–99310 only if F5/F6 is ever served
A2 only, psych	90791 diagnostic evaluation; 90792 with medical services; 90832 / 90834 / 90837 psychotherapy 30/45/60; +90833 / +90836 / +90838 add-on with an E/M; 96127 brief emotional/behavioral assessment; 96116 neurobehavioral status exam
A9, LCSW	90791 (not 90792); 90832 / 90834 / 90837; 90846 / 90847 family psychotherapy; 90853 group; 96127; health behavior assessment codes where applicable. No E/M under any circumstance
Care management (A1, A2, A4 bill)	Part V: 99490, +99439, 99491, +99437, 99487, +99489, G3002/+G3003; 99492, 99493, +99494, 99484

Rules that matter: psych carries standard Part B cost sharing (the outpatient mental health limitation is gone). When psychotherapy and an E/M happen in one encounter, bill the E/M plus the timed add-on, not a standalone psychotherapy code. Psychotherapy time is documented separately from E/M time; the same minutes cannot be counted twice. If any substance use disorder treatment records are held, 42 CFR Part 2 applies on top of HIPAA.

10.2 Modifiers

Configure before the first claim. Without them, legitimate same-day services bundle and deny.

MODIFIER	WHEN
25	Significant, separately identifiable E/M on the same day as another service. AWV plus a problem visit is the routine case; without 25 the E/M denies
95	Synchronous audio–video telehealth

MODIFIER	WHEN
93	Audio-only telehealth, where permitted for behavioral health
59 / X{EPSU}	Distinct procedural service. Sparingly, with documentation
GT	Legacy telehealth on some payers. Confirm per payer

10.3 Fee schedule

Do not set charges equal to the Medicare allowable. A payer never pays more than the billed charge, so charging at Medicare rates caps every commercial and Medicare Advantage payment at Medicare rates, permanently. **Set the charge master at 125 to 150% of the Medicare allowable** (Medicare PFS, Georgia locality, as the reference) and take the contractual writedown at posting. Mark every code active and confirm it appears on a test encounter's Fee Sheet at the charge-master rate. Recurring January task: reload after the annual PFS update.

10.4 Calendar categories

Administration → Clinic → Calendar **VERIFY ON 8.4**: IHPC Initial Visit (H&P) 90 min · IHPC Follow-Up 45 min · CCM Review 20 min · IME / C&P Exam 60 min, distinct colors. Add In Office / Out of Office blocks for each clinician and lay recurring availability into the calendar. The app's scheduling module reads and writes this calendar directly (OpenEMR is the only appointment ledger; the app never models availability). Record each category's numeric id; the encounter category the app uses (`OPENEMR_ENCOUNTER_CATEGORY`, currently 5) must be a real visit category on this instance. **APP**

10.5 Portal off, modules, document routing

- 1 Administration → Config → Portal **VERIFY ON 8.4**: native patient portal disabled and stays disabled.
- 2 Administration → Modules: leave labs and anything unconfigured disabled. Enabled modules are attack surface and audit scope.
- 3 **Document routing rule.** Two document homes, each with a job. Google Drive (BAA): consents, intake paperwork, service agreements, signed care-plan PDFs, anything a client or family views through the portal. OpenEMR Documents: records received for clinical care (discharge summaries, specialist reports, records obtained via ROI) filed to the chart so the clinician has them while charting. The typed chart itself is OpenEMR data either way. Never split one category across both stores. After 8.4 fixes the document endpoint, the app re-enables the OpenEMR copy of signed care-plan PDFs and files received records to the chart (Session 4.5). **APP**

10.6 e-Prescribing enrollment **DEFERRED**

E-prescribing (Surescripts, and EPCS for controlled substances) is out of the app's scope by owner decision. The app records the prescription so the chart is complete; transmission continues as today. Today's only task is starting vendor enrollment (Weno under Administration → Modules, or the chosen vendor) and identity proofing for each prescriber. Configure when it clears. Until prescriptive authority, the GCMB protocol agreement, eRx setup, formulary, and the facility's order process are all confirmed for a clinician, route

medication changes to the patient's established prescriber and document the recommendation, the communication, the acceptance or rejection, and who prescribed.

PART III

Billing

OpenEMR is the billing system of record. Charges, 837P claims through Availity, 835 posting, A/R, and statements live here. The app is the capture surface: clinicians never open OpenEMR to code a visit. The two meet at the charge-ready gate, which the app enforces as its sign-and-close contract and OpenEMR carries as evidence.

Phase II · Payers and Availity

MONDAY · 1.5 HRS + ENROLLMENT LEAD TIME

II.1 Insurance companies

Administration → Practice Settings → Insurance Companies **VERIFY ON 8.4** → Add a Company. One record per payer: name, claims address, and the payer ID **from Availity's payer list** (Availity Essentials → Payer List), never from the payer's website.

Priority order for the current book: Medicare Part B (Palmetto GBA, Jurisdiction J) first; then each Medicare Advantage plan on the roster as its own payer; then Medigap carriers; then Georgia Medicaid. **Commercial payers are entered only when a real patient carries them and GFC holds a contract and credentialing with that payer.** Adding a payer to OpenEMR does not make you able to bill it. Commercial credentialing runs through CAQH ProView, one profile per clinician, kept current.

II.2 X12 partner

- 1 Administration → Practice Settings → X12 Partners **VERIFY ON 8.4** → Add New Partner. Name Availity. Version 005010X222A1 (837P).
- 2 Sender IDs (ISA06/GS02): the Availity Customer ID issued at registration. Receiver IDs and qualifiers (ISA05/07/08, GS03): copy exactly from the Availity EDI Connection Guide. Do not guess.
- 3 Reopen each payer from 11.1 and set its X12 Partner to Availity.

II.3 Availity enrollment **OWNER**

In enrollment since July; do not switch. Register Availity under GFC's group NPI and EIN once the 855B is approved, not under an individual. EDI enrollments through Availity with Palmetto: 837 claims, 835 ERA, 270/271 eligibility. Medicare and Medicaid ERA authorizations run multi-week; they are the longest pole in the billing build and none of it blocks the rest of this guide. **Execute a BAA with Availity** before the first claim; they handle every claim's worth of PHI.

What "Availity SFTP" is and is not

Availity SFTP is a file-transfer channel: an automated drop of 837 batches and pickup of 835 files instead of clicking upload/download in Essentials. It is not eligibility. Eligibility is the 270/271 transaction, which runs today by hand in the Essentials portal and later through Availity's real-time API from the app (Track D, B1). SFTP automation waits for volume (Appendix D).

Phase 12 · Per-patient setup

BEFORE EACH FIRST VISIT · 15 MIN PER PATIENT

The app creates or links the OpenEMR patient record at enrollment (FHIR Patient create, one record in OpenEMR, the app stores only the pointer). Demographics come from the app's intake. The billing fields below are entered in OpenEMR by staff until the app's B-series carries them. **APP**

- Name exactly as on the Medicare card, DOB, residence address.
- Insurance: MBI primary; Medigap or Medicaid secondary; subscriber equals self.
- Signature on file YES for release of information and assignment of benefits, backed by the intake packet the patient or legal representative signed in the app (consentToTreat, assignmentOfBenefits, practiceNpp are captured at intake; paper-signed packets carry `signed_offline`). A facility nurse supervisor cannot sign unless she holds documented legal authority.
- POA or responsible party recorded, with a separate billing contact. The app's `familyIsPoa` flag is set only after the POA document is verified on file.
- Existing PCP recorded.
- Facility record assigned from 7.4, which drives POS.

Keep these as separate fields, since each drives different billing behavior: Medicare Advantage enrollment and plan name; Medicaid eligibility and category; QMB status; secondary or Medigap carrier. Every QMB is dual eligible; not every dual is QMB.

Two revenue assumptions to verify per patient

Annual wellness visits. G0438 is generally once per lifetime, G0439 once per 12 months, and payment follows the beneficiary's claims history, not which practice considers itself primary. Pre-check last G0402, G0438, G0439 dates in the Palmetto eServices portal; a 270/271 alone will not reliably return AWW history. Keep AWW out of any launch revenue forecast until the roster is checked.

Medicare Advantage. MA determines whether a visit is billable at all. Sort every roster into: Original Medicare FFS; MA in-network; MA out-of-network but potentially payable; MA requiring authorization or a single-case agreement; unverifiable. For an out-of-network MA patient a claim to Palmetto is not payable. MA behavioral health often carries separate network and authorization rules from medical; check both lanes.

Phase 13 · Visit workflow, charge capture, and the charge-ready gate

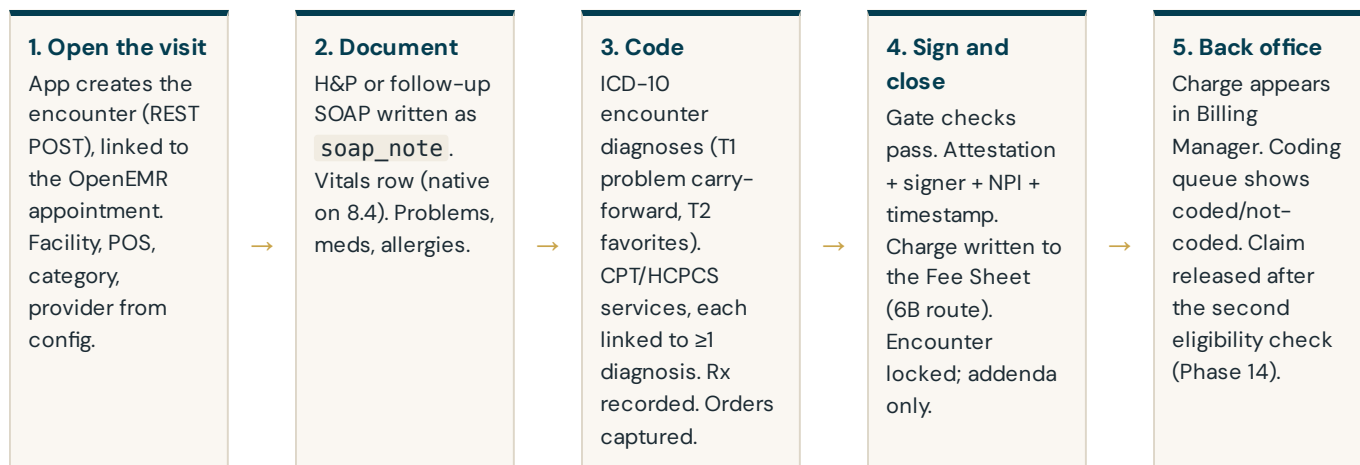
EVERY VISIT

13.1 Encounter defaults

Service facility per 7.4, billing facility GFC, rendering provider set to the person in the room. Defaults must be overridable. Each note carries: service date, patient location, rendering clinician; medically appropriate history and exam; either the MDM basis or a total practitioner time statement (psych documents psychotherapy time

separately from E/M time); assessment and plan tied to each diagnosis on the claim; medication reconciliation and where prescribing was routed; the rendering practitioner's own signature and attestation; a late-entry policy preserving true date, time, and amendment reason.

13.2 The charge path as the app runs it APP



Rendering provider on the charge is the signing clinician, stamped with the NPI on their app user record. Billing provider is the config value `gfc_payer_credentialing.billing_npi_used`, edited by an admin at `/clinical` → Coding queue → Billing settings, never a literal in code. Signing is blocked without it, on purpose.

13.3 The charge-ready gate

Nothing enters the billing queue until all eight are true. The app enforces what it can see; OpenEMR and the staff carry the rest.

#	CONDITION	ENFORCED WHERE TODAY	APP BLOCK CODE
1	Note signed and locked	App sign-and-close; encounter becomes read-only, corrections as addenda with their own signer and timestamp	<code>SIGN_NO_NOTE</code>
2	Diagnosis-to-service linkage reviewed	App: ≥1 ICD-10 diagnosis, ≥1 service, every service linked	<code>SIGN_NO_DIAGNOSIS</code> <code>SIGN_NO_SERVICE</code> <code>SIGN_UNLINKED_SERVICE</code>
3	Rendering provider validated against the roster	App: signer must have an NPI on file (flagged for test use); OpenEMR: provider record exists with Provider + Authorized	(attestation flag)
4	Rendering provider's certification matches the code billed	Not yet in the app. Back-office check against the 7.3 table until Session 4.5 adds a permitted-code-set per archetype on the user record. A1 cannot bill the psych set; A9 cannot bill E/M; A10 bills nothing independently	(4.5 scope)
5	Facility record and POS match where the service happened	OpenEMR encounter record; app default is F1 until 4.5 adds the per-visit facility picker (8.3)	—

#	CONDITION	ENFORCED WHERE TODAY	APP BLOCK CODE
6	Eligibility and payer order validated	Staff, Phase 14, twice per claim; app surfaces coverage, MA status, and the QMB flag at intake	—
7	Enrollment and reassignment active for that date of service	Staff, against PECOS status. Hold the claim otherwise	—
8	Required consent, notice, or authorization documented	App intake consents; ABN where coverage is doubtful (Part VI C-6)	—
—	Billing NPI set	App	SIGN_NO_BILLING_NPI

Requiring codes at sign is why this works: no encounter goes unbilled, and nothing bills that should not. The attestation text, signer name, credential, NPI, and timestamp live on the app's `encounter_attestations` record and in the `[GFC ATTESTATION]` block of the structured note in the chart, because OpenEMR's API has no sign concept (Part VII.6).

Phase 14 · Eligibility, twice per claim

BEFORE THE VISIT · BEFORE RELEASE

Run 270/271 before the visit and again before claim release. Enrollment changes between those points, and the second check is what stops a claim going to Palmetto for a patient who moved to an out-of-network MA plan last month.

STAGE	NOW (LAUNCH)	LATER (TRACK D, B1)
Pre-visit	Staff run the patient in Availity Essentials → Eligibility and Benefits. Record: coverage active, MA plan and network, QMB, Medigap/Medicaid secondary, deductible remaining. Update the OpenEMR insurance record and the app's payer fields.	App calls Availity's real-time 270/271 API from the client profile at intake and again before each visit; results land on the client record and drive the QMB hard hold automatically.
Pre-release	Repeat in Essentials for every encounter in the batch. Any change: fix the payer record, re-run the Billing Manager claim.	Batch eligibility from the coding queue before release.

Neither stage is Availity SFTP. SFTP moves claim and remittance files; it never answers "is this patient covered today."

Phase 15 · Claims out

WEEKLY BATCH · 5 MIN PER BATCH

- 1 Fees → Billing Manager `VERIFY ON 8.4`. Filter to encounters that are coded and signed (the app's coding queue shows the same list). Validate payer IDs, provider IDs, facility and POS mapping, and the rendering field.
- 2 Generate the 837 batch. Download the X12 file.
- 3 Availity Essentials → Claims & Encounters → EDI upload. Upload the file.

- 4 **The first time:** send one test claim, confirm acceptance (999 accepted, 277CA accepted), then release the rest.
- 5 Mark the batch as billed in Billing Manager so the encounters leave the queue.

Keep three failure modes separate

FAILURE	SIGNAL	OWNER	FIX
Clearinghouse rejection	Availity 999 / front-end edit	A6 or A7	Format or field error; correct in OpenEMR, regenerate
Payer front-end rejection	277CA rejected	A6 or A7	Identifier problem (NPI, taxonomy, address, enrollment); usually a Phase 7 item, not a coding item
Adjudicated denial	835 with a denial reason (CARC/RARC)	A7 with the clinician	Coverage, medical necessity, bundling, modifier; correct and resubmit, or appeal

Timely filing: 12 months for Medicare, often 90 to 180 days commercial. Track per payer.

PART IV

Remittance and deposits: the operating procedure

This is the part the three source guides never wrote. It defines what "necessary" means for getting paid, who does it, and how often. It is deliberately small. Everything beyond it has a trigger in Appendix D.

Phase 16 · From claim to bank

WEEKLY AND MONTHLY · A6 NOW, A7 WHEN HIRED

16.1 The money path

1 · 837 out Phase 15. Batch from Billing Manager, uploaded to Availity.	2 · Payer adjudicates Palmetto (or the MA plan) allows, pays, applies deductible and coinsurance, adjusts.	3 · Two things arrive separately The 835 ERA (the explanation, a file, via Availity) and the EFT (the money, into the bank). Same trace number on both.
4 · Post the 835 In OpenEMR. Allowed, paid, adjustment, and patient responsibility land on each encounter.	5 · Patient responsibility Routed per 16.4: crossover, Medicaid, QMB hold, or a statement.	6 · Reconcile Bank deposit equals 835 paid total, matched by trace number. Weekly.

Two rules that carry the whole part

Patient responsibility comes from the 835, never from a hand calculation off the charge master. And no statement, charge, or collection task goes out before the ERA posts.

16.2 One-time setup

#	ITEM	WHERE	NOTES
1	Dedicated clinical operating bank account	Bank OWNER	Separate from the home-care account. This is the "separate books by line" mitigation from 7.2 and it is what makes deposit reconciliation possible. All Part B EFTs and all patient payments for the clinical line land here.
2	Medicare EFT authorization (CMS-588)	PECOS, with the 855B OWNER	Medicare pays by EFT only. The 588 binds the clinical account to the group enrollment. Bank letter or voided check required. Changes to the account later mean a new 588.
3	835 ERA enrollment per payer	Availity → Payer Enrollment OWNER	Medicare and Medicaid ERA authorizations run multi-week; MA plans vary. Until a payer's ERA enrollment is active, its remittance arrives on paper and is posted as a manual EOB (16.3 step 6).
4	Remittance screens confirmed	OpenEMR: Fees → Posting Payments (835 upload) and Fees → Batch Payments (manual EOBs) VERIFY ON 8.4	Both open under the admin login and under the Accounting ACL group. Clinicians do not see them (8.5).
5	QMB hard hold	OpenEMR patient insurance record + app payer flag APP	For confirmed QMB patients, statements, collection tasks, and manual balance transfers are blocked. In OpenEMR today this is the patient's billing note plus staff discipline; the app enforces it as a system control at B2/B5. Federal law prohibits billing QMB individuals for Medicare-covered cost sharing, including under MA and including when Medicaid pays less than the full amount.
6	Write-off, adjustment, and refund policy	Written document OWNER	Who may adjust, at what dollar threshold, with what documentation. Required before the first posting so that contractual adjustments are distinguishable from discretionary ones.
7	Statement template and delivery	OpenEMR statements at launch; app + Stripe later	OpenEMR generates the statement; it is mailed or emailed (Workspace Gmail, BAA) by staff. The app's statement trigger and Stripe collection are B2/B5 and fire only on posted-ERA patient responsibility read from OpenEMR.

16.3 Weekly: post remittances

Every Friday, before the denial report. Manual posting for the first months is deliberate: it is how the practice learns each payer's adjustment and denial patterns before trusting automation.

- Download.** Availity Essentials → Claims & Encounters → Remittance Viewer (or EDI download) → each new 835 file. Save to the clinical line's remittance folder in Workspace Drive (BAA), named by payer and check date.

- 2 **Upload to OpenEMR.** Fees → Posting Payments → upload the 835 VERIFY ON 8.4. OpenEMR matches each claim line to the encounter and shows allowed amount, paid amount, deductible applied, coinsurance, contractual adjustment, and any denial codes.
- 3 **Review before posting.** For each line: paid amount is plausible against the Georgia PFS allowable; the contractual adjustment is the difference between charge and allowed (expected, from the 125 to 150% charge master); anything else (a denial, a partial, a takeback) is flagged, not posted as a write-off. Unmatched lines (a claim the payer paid that OpenEMR cannot find) are investigated the same day.
- 4 **Post.** Payments and adjustments land on the encounters. Patient responsibility is now visible per encounter.
- 5 **Record the trace number.** Each 835 carries a trace number (the TRN segment) that also appears on the bank's EFT. Note it with the 835 total in the reconciliation sheet (16.5).
- 6 **Paper EOBs.** For payers without ERA enrollment yet: Fees → Batch Payments, enter the EOB by hand with the same review discipline. Scan the EOB to the remittance folder.

16.4 Patient responsibility routing

2026 figures: \$283 annual Part B deductible, then Medicare pays 80% of the approved amount and 20% is patient responsibility. Psych is treated the same as medical. No cost sharing on AWWs (G0438, G0439) or on ACP (99497) furnished with a covered AWW.

PATIENT SITUATION	WHO PAYS THE 20%	STATEMENT?	ACTION AFTER POSTING
Medigap	Medigap, via automatic crossover	No	Confirm the crossover on the first remittance rather than assuming it. If the secondary does not pay within 30 days, drop the secondary claim manually.
QMB	Nobody bills the patient. Write off what Medicaid does not pay	Never	Hard hold. Bill Georgia Medicaid as secondary; adjust the remainder with the QMB reason code.
Medicaid, non-QMB	Medicaid up to its allowable	No	Bill Medicaid secondary; adjust the remainder.
Traditional Medicare, no secondary	Patient or POA	Yes	Statement from OpenEMR to the billing contact on the patient record, only after the ERA posts. Later: app reads the balance via API and collects through Stripe (B2/B5).
Medicare Advantage	Plan copay, per the plan's EOB	Per plan	Fixed-dollar copays defined by the plan may be collected at the visit; everything else waits for the EOB.

Recommended default for Medicare patients: collect nothing at the visit and bill after the ERA posts. Collect at time of service only where a plan defines a fixed dollar copay. This overrides the earlier "charge estimated copay at time of service" note in the app spec; the 125 to 150% charge master would overcharge every patient if the copay were computed from it.

16.5 Weekly: deposit reconciliation

One sheet, one row per remittance, kept in the clinical line's finance folder. Columns: payer, check/EFT date, trace number, 835 paid total, bank deposit amount, bank posting date, difference, status. The rule is simple: every bank deposit into the clinical account matches an 835 by trace number and amount, and every posted 835 matches a deposit.

- 1 Export the week's deposits from the clinical bank account.
- 2 Match each EFT to a posted 835 by trace number. Amounts must agree to the cent.
- 3 Unmatched deposit: a remittance you have not downloaded or posted yet. Find it in Availity and post it. Unmatched 835: money not yet in the bank (normal for a day or two) or an EFT routed elsewhere (a 588 problem). Escalate after five business days.
- 4 Patient payments (checks, later Stripe payouts) reconcile the same way against the patient-payment entries posted in OpenEMR. When Stripe lands, its payout report is the "835" for patient money: one payout, one row, matched to the app's payment records that were written back to OpenEMR's ledger. The app never holds a second balance.
- 5 Sign and date the week's sheet. Second-person review monthly (16.7).

16.6 Weekly: denials and rejections

Work the report every Friday after posting. Three buckets from Phase 15 (clearinghouse, payer front-end, adjudicated), each with its owner. For adjudicated denials: read the CARC/RARC codes, decide correct-and-resubmit versus appeal versus write-off under the policy, and act inside the payer's timely-filing and appeal windows. Log every denial with reason code, action, and outcome; after a few months the log tells you which Phase 7 and Phase 10 items need fixing.

16.7 Monthly close

TASK	OWNER	DETAIL
Credit balance report	A6 / A7	Run OpenEMR's credit balance report. Medicare overpayments must be reported and returned within 60 days of identification. Identify, investigate, refund or report, with a named owner and a dated log. The clock starts at identification.
Adjustment and write-off review	Second person (not the poster)	Review every non-contractual adjustment against the policy. Logged with findings. This is the compensating control for A6 holding clinical, billing, and admin access at once.
A/R aging	A6 / A7	Insurance A/R by payer and age bucket; patient A/R by age. Anything over 60 days insurance is a follow-up task; over 90 days is an escalation.
Deposit sheet sign-off	A6	All weeks matched; open items carried with a date.
Books	Bookkeeper	Clinical account activity to the accounting system by line. Whether the app feeds an accounting system is an open consultant item; until decided, the deposit sheet is the bridge.

16.8 The scope boundary

Necessary at launch

Dedicated clinical bank account. CMS-588 EFT. ERA enrollment per payer through Availity. Manual 835 posting weekly with review before posting. Patient responsibility routed by the 16.4 table with the QMB hold. Weekly deposit reconciliation by trace number. Weekly denial work. Monthly credit balance, adjustment review, aging, and sign-off. A written write-off policy. Owner named for each.

Deliberately later, with triggers

ERA auto-posting: after several clean months. Availity SFTP for file movement: at volume. App-read of patient balances and Stripe collection with write-back to OpenEMR's ledger: Track D B2/B5, after Session 5 go-live. Automated eligibility (B1): when the app's payer screen is ready. Accounting-system feed: when the consultant answers whether one exists. None of these change the rules above; they change who clicks.

PART V

Care management line (CCM, CoCM, BHI)

Live at launch on an external care management platform, integrated with OpenEMR. OpenEMR stays the billing system of record: every charge is entered here because the 837 is generated here.

V.1 The split

SYSTEM	ROLE
External platform	Registry, comprehensive care plan, time tracking, outreach logging, task queues. The system the care team works in daily.
OpenEMR	Billing system of record. Every charge lands here, transmitted through Availity. A platform that bills independently would split the claim stream and break reconciliation, denial tracking, and ERA posting.
GFC app	Not in the loop for care-management billing today. The app's own care plan (RN-authored, client co-signed, signed PDF) is the PHCP/clinical care plan for the household; it is not the CCM comprehensive care plan unless the platform decision says so.

Outbound, OpenEMR → platform: roster, demographics, insurance and payer flags, problem list, medication list, allergies, encounter history, as FHIR reads through an OAuth2 client of the platform's own (never the app's client). **Inbound, platform → OpenEMR, monthly per patient:** the CPT code earned, supporting ICD-10 codes, service date, rendering practitioner, total qualifying minutes, plus the care plan document and time-log summary filed to the chart. Encounter and document writes ride the Standard REST lane; the charge itself uses the 6B fee-sheet route or is keyed in Billing Manager until then. The time log stays in the platform but must be exportable and retained on GFC-controlled storage; in a CCM or CoCM audit it is the evidence.

V.2 Codes

CODE	WHOSE TIME	THRESHOLD
99490 / +99439	Clinical staff	First 20 min / each additional 20
99491 / +99437	Practitioner only	First 30 min / each additional 30
99487 / +99489	Clinical staff, complex	First 60 min / each additional 30
G3002 / +G3003	Practitioner	Chronic pain management
99492 / 99493 / +99494	CoCM	Initial month 70 min / subsequent 60 / each additional 30
99484	General BHI	20 min

Clinical-staff codes include both staff and practitioner time; practitioner codes count only the practitioner's own minutes. Do not mix them for the same patient in the same month.

V.3 Enrollment gates, per patient

1. Two or more chronic conditions expected to last at least 12 months or until death, placing the patient at significant risk.
2. Initiating visit completed (comprehensive E/M, AWW, or IPPE) for new patients or anyone not seen in 12 months, with the CCM discussion inside that visit. The Hickory Log first visits are the initiating visits; the consent conversation belongs in that visit script, so build consent capture into the encounter template now.
3. Consent documented, written or verbal: the service is available, cost sharing applies, only one practitioner may bill CCM per calendar month, they may stop at any month end. Record accepted or declined either way.
4. No other practitioner is already billing CCM for that patient. Verify before enrolling.
5. Comprehensive electronic care plan created and available inside and outside the practice.
6. 24/7 access in place: a designated care team member, more than one contact method. Decide who answers at 2am before the first patient enrolls.

V.4 Concurrency and staffing

One practitioner per patient per month for CCM. CCM and CoCM/BHI may be billed in the same month but no minute counts twice; every logged activity belongs to exactly one program, so the platform must separate time at the activity level. Complex and non-complex CCM never in the same month. CCM not alongside home health or hospice supervision or ESRD services. Complex CCM not with prolonged E/M. Clinical staff time runs under general supervision (the billing practitioner directs but need not be present), which is what makes an LMSW's time billable here. The billing practitioner on every care-management claim is A1, A2, or A4. Monthly reconciliation: platform-eligible patients against charges posted in OpenEMR; a gap in either direction is a revenue leak or a compliance problem. Cost sharing is a patient conversation: 20% coinsurance monthly is a real bill for a traditional-Medicare patient with no secondary, and nothing for QMB.

PART VI

Compliance controls and calendar

The July guide had fourteen gaps, four with real financial or legal exposure. They are folded here as numbered controls with an owner and a cadence, so they stop being an audit finding and become a routine.

#	CONTROL	WHAT IT IS	CADENCE
C-1	Exclusion screening	Every clinician, employee, and contractor screened against OIG LEIE and SAM.gov at hire; full roster re-screened monthly; dated results retained. Hard gate in the 7.3 onboarding checklist. Highest-exposure gap found in the audit.	Hire + monthly
C-2	Credit balances and overpayments	Monthly report; identify, investigate, refund or report within 60 days; named owner (Phase 16.7).	Monthly
C-3	Write-off, adjustment, refund policy	Written thresholds and approval path; monthly adjustment report reviewed by someone other than the poster.	Monthly
C-4	Segregation of duties, compensating control	A6 holds clinical, billing, and admin access and is the owner. Acknowledged; compensated by C-3's second-person review and C-8's quarterly audit-log review with written findings.	Monthly + quarterly
C-5	Credential expiration tracking	Licenses, certifications, DEA, PDMP, CAQH attestation, malpractice, protocol agreements, with expiration dates in one tracker. A lapsed credential mid-period makes every claim in the window suspect.	Annual sweep, monthly glance
C-6	ABN process	Advance Beneficiary Notice stocked, staff trained on when to issue, signed copies filed to the chart. Without it GFC absorbs the denial.	Per visit as needed
C-7	BAAs	AWS (done), Availity, each facility where GFC handles its PHI, the care-management platform, the telehealth platform before psych go-live, Google Workspace (existing). Stripe is the intentional exception through PHI segregation.	Before PHI moves
C-8	Audit log review	Administration → System → Logs reviewed quarterly with written findings. Not "glance at it."	Quarterly
C-9	Secrets and access hygiene	Rotate DB-MASTER, DB-OPENEMR, OE-ADMIN before Session 5. MFA on every OpenEMR account and on the AWS account. No shared logins. Disable the password grant and retire <code>gfc-app-api</code> at Session 5. Real secrets only in the password manager and Secrets Manager.	Before go-live, then per change

#	CONTROL	WHAT IT IS	CADENCE
C-10	Server and application patching	Monthly: <code>apt-get upgrade</code> ; <code>docker compose pull && up -d</code> on the pinned line; re-apply and re-test the 6B route patch (or rebuild the derived image); confirm yesterday's EBS snapshot and RDS backups exist; check the budget.	Monthly, first Monday
C-11	Restore drill	Restore the newest RDS snapshot to a scratch instance, confirm it opens, delete the scratch. First drill before Session 5 go-live. A backup never restored is a hope.	Quarterly
C-12	Delegating physician chart review	Per 7.5: controlled-substance records quarterly, adverse outcomes within 30 days, 10% of the rest annually. Documented.	Quarterly
C-13	Fee schedule and code set reload	Reload the fee schedule after the annual Medicare PFS update (January); reload ICD-10-CM after the October 1 release.	Annually, twice
C-14	Facility POS re-verification	Against current DCH licenses.	Annually
C-15	Medicare revalidation	Every five years; PECOS notices go to the contact on file, so that contact must be monitored.	Every 5 years
C-16	Record retention and Part 2	Retention schedule reconciling Georgia requirements with Medicare's. 42 CFR Part 2 analysis before any SUD records are held.	Once, then per change
C-17	Telehealth establishment dates	Medicare telehealth flexibilities run through 12/31/2027; from 01/01/2028 behavioral telehealth needs an in-person visit within six months prior to the first service and annually after, with patients established by 12/31/2027 grandfathered. Track per patient. Confirm with Palmetto whether a resident of a licensed community counts as "home" for POS 10.	Per patient

VI.1 The calendar

CADENCE	TASK
Weekly (Fri)	Post remittances (16.3) · deposit reconciliation (16.5) · denial and rejection report worked (16.6)
Monthly	OIG/SAM re-screen (C-1) · credit balance report (C-2) · adjustment review by second person (C-3) · patch, backup verification, cost check, re-apply 6B patch (C-10) · A/R aging and close (16.7)
Quarterly	Delegating physician chart review (C-12) · restore drill (C-11) · audit log review with written findings (C-8)
Annually	Credential sweep (C-5) · January fee schedule reload and October ICD-10 reload (C-13) · facility POS re-verification (C-14)
Every 5 years	Medicare revalidation (C-15)

PART VII

App alignment

The GFC Care Platform is the front end. OpenEMR is the clinical and billing system of record. This part is the contract between them as of Session 4.4 merged, and it records what 8.4 changes so Session 4.5 wires the native paths and retires the workarounds in the right order.

VII.1 Division of labor

- One patient record, in OpenEMR. The app stores only the pointer (`client.openEmrPatientId`), the app-authored care plan and its versions, enrollment and consent state, and activity entries.
- Clinicians and families never open OpenEMR. The app reads and writes through the API. Bethel's OpenEMR login exists for the Sunday-night fallback and for back-office screens, not for charting.
- The app never keeps its own money ledger or its own code list. Balances come from OpenEMR; codes come from OpenEMR. If they ever disagree, OpenEMR wins.
- Every EMR call the app makes is logged with the acting app user, role, patient, resource, and action. Until Session 5 this is the attribution trail; OpenEMR itself sees `gfc-app-api`.

VII.2 The two-lane API on 8.4

Reads ride FHIR R4 (`/apis/default/fhir/...`): Patient, Condition, MedicationRequest, AllergyIntolerance, Encounter, CarePlan, DocumentReference, Observation, Practitioner. Writes ride the Standard REST lane (`/apis/default/api/...`): encounter, soap_note, vital, medical_problem, medication, allergy, appointment, document, and on 8.4 prescription, plus the two 6B routes. Patient create and update go FHIR. Anything coded FHIR-only stalls on visit documentation.

Instance quirks the app already encodes (carry forward, re-verify on 8.4)

- `soap_note`, `vital`, `medication`, and every appointment endpoint key by **numeric** pid and encounter id; uuids are silently coerced to 0. The app resolves ids first and caches the encounter id from the create response. This is the defect that orphaned every 4.1 note at patient zero, fixed in PR #29.
- The SOAP-note validator answers HTTP 200 with a validation map when a section is under two characters and writes nothing; the app treats a missing `sid` as a rejection. The note list joins without a `formdir` filter and can leak another encounter's note; the app reads notes back by the `sid` it recorded.
- FHIR Patient POST returns `{pid, uuid}` rather than the resource; both encounter list endpoints return every row twice (deduped by id); the standard-API medication list GET 500s on uuid-less rows, so med lists are read via FHIR MedicationRequest; `medical_problem.begdate` is a plain date while `allergy.begdate` is a datetime.
- No appointment update in any shape: reschedule, cancel, and no-show are a tombstone swap (replacement rows posted first, superseded row removed last, nothing ever leaves the calendar). The list endpoints omit `pc_hometext` / `pc_duration` / `pc_room`; only the single-row GET returns them. Location and telehealth flag ride as a `[GFC location=...]` line in `pc_hometext`.
- Encounter POST accepts `billing_note`; the encounter PUT 500s until the `sensitivities` ACL is added (8.6). `GET /fhir/Encounter/{id}` 403s on the org-policy ACL while the search works.

VII.3 Configuration values the app consumes

APP SETTING	WHERE IT COMES FROM IN OPENEMR	CURRENT / TARGET
OPENEMR_BASE_URL , OPENEMR_SITE	Phase 9.1 site address; site <code>default</code>	https://emr.godwinsfamilycarellc.com / default
OPENEMR_CLIENT_ID , OPENEMR_CLIENT_SECRET	Phase 9.3 registration, Secrets Manager <code>gfc/openemr/oauth-client</code>	v2 client (4.2 scopes) → v3 client (8.4 scopes) after 6A
OPENEMR_API_USERNAME , OPENEMR_API_PASSWORD	Phase 9.4 <code>gfc-app-api</code> user	Dev window only; retired at Session 5
OPENEMR_SCOPES (optional override)	Phase 9.3 scope string	Defaults to <code>config.js</code> list; set only if the registered set differs
OPENEMR_FACILITY_ID	Phase 8.3 facility record numeric id (F1)	3 today; confirm after facility rebuild
OPENEMR_POS_CODE	Phase 8.3 F1 POS	12; per-visit facility picker in 4.5 replaces the single default
OPENEMR_ENCOUNTER_CATEGORY	Phase 10.4 calendar/encounter category id	5; confirm it is a real visit category on 8.4
OPENEMR_PROVIDER_ID	Phase 8.4 provider numeric id	1 (installer admin) → Bethel's id; the signer's <code>openEmrProviderId</code> should override per encounter in 4.5
User <code>openEmrProviderId</code> (admin-hub user form)	Phase 8.4 provider numeric id per clinician	Required before a clinician can see or write their calendar (409 <code>PROVIDER_NOT_MAPPED</code> otherwise)
User <code>npi</code> (admin-hub user form, 10 digits)	Phase 8.4 provider NPI, same value	Drives note attribution and the rendering provider on every charge
<code>gfc_payer_credentialing.billing_npi_used</code> (Coding queue → Billing settings; env <code>GFC_BILLING_NPI_USED</code> seeds it once)	Phase 8.2 organizational NPI once the 855B lands; Bethel's individual NPI until then	Empty today; signing blocked until set. Flip to the Type 2 clinical NPI the day the facility NPI field is populated, same day, both places
Admin practice service favorites (Coding queue)	Phase 10.1 fee schedule	Must mirror the fee schedule exactly: 99341/42/44/45, 99347–50, family labels only, no AMA descriptors. Add G0438/G0439/99497/99495/99496 when Dianne confirms
Appointment categories the scheduler offers	Phase 10.4 calendar categories	Read live from OpenEMR (<code>user/list.read</code> on 8.4)

VII.4 Compatibility matrix: 7.0.4 verified → 8.4 expected

Sign the last column only after testing against the live 8.4 server with TEST PatientOne. This table replaces Appendix D of the July guide.

VISIT ACTION	LANE AND ENDPOINT	7.0.4 VERIFIED STATE	8.4 EXPECTED	VERIFIED
Patient list, chart demographics	FHIR GET Patient	Works	Same	☐
Create / link patient	FHIR POST/PUT Patient	Works; POST returns {pid,uuid}	Same; re-check response shape	☐
Problems, meds, allergies, vitals, encounters, care plan, documents into the chart view	FHIR GET Condition, MedicationRequest, AllergyIntolerance, Observation, Encounter, CarePlan, DocumentReference	Works; Condition drops ICD coding until the code set is loaded; encounter rows duplicated	Coding present after 8.6 load; re-check duplication	☐
Start a visit	REST POST patient/{puuid}/encounter	Works; accepts billing_note	Same; PUT works once sensitivities is granted	☐
H&P / follow-up SOAP note	REST POST/PUT/GET patient/{pid}/encounter/{eid}/soap_note	Works with numeric ids; 200-with-validation-map quirk	Same; re-check both quirks	☐
Vitals	REST POST patient/{pid}/encounter/{eid}/vital	500 unconditionally (null authUserId); readings preserved in the note	Fixed. Re-enable the native write; keep the note copy of both-arm BP by design	☐
Problem list write	REST POST/PUT medical_problem	Works	Same	☐
Medication list / reconciliation	REST POST/PUT patient/{pid}/medication; read via FHIR MedicationRequest	Works	Same	☐
Allergy	REST POST allergy	Works	Same	☐

VISIT ACTION	LANE AND ENDPOINT	7.0.4 VERIFIED STATE	8.4 EXPECTED	VERIFIED
Prescription record	8.4: REST POST /api/prescription	No write route; app-side prescriptions record + medication-list row with the sig in the title + [GFC RX] block	Native. Switch in 4.5; keep the medication-list row only if the prescription does not surface in MedicationRequest	☐
Orders (lab, imaging, procedure)	6B route over ProcedureService	No write route; app-side <code>clinical_orders</code> + [GFC ORDERS] block; status advanced manually	Patched route. Switch in 4.5; status workflow stays app-side until 8.4's procedure update path is verified	☐
Fee-sheet charge (CPT + ICD pointers + modifiers + provider)	6B route over BillingUtilities::addBilling()	No route at all; app-side <code>encounter_billing</code> + [GFC CODING] block; back office keys it	Patched route. Charge lands in Billing Manager at sign; retire the app-side store in a second commit	☐
Encounter sign / close, addenda	No API concept	App-side <code>encounter_attestations</code> , <code>encounter_addenda</code> , [GFC ATTESTATION] and [GFC ADDENDA] blocks	Unchanged. Stays app-side by design	☐
Code search (ICD-10, CPT)	None in either API; FHIR ValueSet serves list_options only	Picker fed from problem list, clinician history, admin favorites, typed full code	Optional 6B code-search route retires the last workaround	☐
Appointments: list, create, tombstone swap	REST GET/POST/DELETE patient/{pid}/appointment	Works; no PUT/PATCH; list omits fields	Re-check whether 8.4 added an update route; if so, the swap stays as the safe path until proven	☐
Providers list	FHIR GET Practitioner; REST GET practitioner	403 on org-policy ACL; app falls back, flagged degraded	Works after 8.6 ACL grant	☐
Documents: signed care-plan PDF, received records	REST POST/GET patient/{puuid}/document?path=...	500 (SQL bind bug); Drive copy only	Fixed. Re-enable the OpenEMR copy and the received-records filing per the routing rule	☐

VISIT ACTION	LANE AND ENDPOINT	7.0.4 VERIFIED STATE	8.4 EXPECTED	VERIFIED
Coding queue (whole-instance encounters)	FHIR GET Encounter?_count=200 paged	Works	Same	☐
Patient portal clinical read (4.3)	Scoped FHIR reads brokered by the app server	Not built	Same lane; serve the care-plan PDF from Drive; exclude tombstoned appointments	☐
Billing read-through to the portal, payment write-back	Track D B5, not built	—	Reads posted-ERA patient responsibility; writes Stripe payments back to the ledger	☐

VII.5 What Session 4.5 retires, and in what order

Do not retire an app-side store in the same commit that adds its native write. Add the native path, prove it against the live EMR with the round-trip harness (the 4.4 build has a 91-check live harness to extend), then remove the workaround in a second commit. Two changes at once make a failure impossible to attribute.

WORKAROUND TODAY	NATIVE PATH ON 8.4	RETIRE
<code>encounter_billing</code> KV + <code>[GFC CODING]</code> block; back office keys the charge	6B fee-sheet route at sign-and-close	Second commit, after Billing Manager shows the charge for a signed TEST encounter
<code>clinical_orders</code> KV + <code>[GFC ORDERS]</code> block	6B order route	Second commit; keep the app status workflow (ordered → sent → resulted / cancelled) until an update path is verified
<code>prescriptions</code> KV + medication-list row with packed sig + <code>[GFC RX]</code> block	<code>POST /api/prescription</code>	Second commit; confirm read-back through FHIR MedicationRequest or the prescription GET first
Vitals preserved in the note with a warning	Native vital row	Immediately after the vitals POST returns a row; the both-arm BP text stays in the note by design
Documents: Drive copy only, OpenEMR copy skipped and logged	Native document upload	Re-enable; Drive remains the portal-facing home per the routing rule
Provider list degraded fallback	FHIR Practitioner after the ACL grant	Keep the fallback code path; it costs nothing and covers the next ACL regression
Single facility/POS default from env	Per-visit facility picker sourced from OpenEMR's facility list	New in 4.5; POS inherits from the facility record

WORKAROUND TODAY	NATIVE PATH ON 8.4	RETIRE
Certification-to-code check done by back office	Permitted code set per archetype on the app user record, enforced at sign	New in 4.5 if time allows; otherwise 4.6

VII.6 What stays app-side, by design

- Sign-and-close, attestation, and addenda (`encounter_attestations`, `encounter_addenda`, and their note blocks). OpenEMR's API has no sign concept; the structured note carries the attestation into the chart so it is visible in OpenEMR too.
- The clinician's own coding favorites (T2 usage rows) and the admin practice favorites. These are picker conveniences, not a code list; the codes themselves are validated against OpenEMR.
- Appointment ↔ encounter pointers (`appointment_encounters`), consent events, care-plan versions and co-sign events, enrollment checklist stamps, activity log. None of these is clinical content that belongs in the chart.
- `gfc_payer_credentialing`, including `billing_npi_used` and, at the B-series, per-payer credentialing status that decides claim routing (in-network or out-of-network → 837 via Availity; pending → hold; not credentialed → self-pay + superbill).
- Attribution stamps in note headers and encounter fields until Session 5's per-user auth makes OpenEMR's own signature match the claim.

APPENDICES

A · Acceptance test

Run before the first real claim. All must pass. This merges the July end-of-day test, the v3.0 acceptance test, and the Runway's "walk one full visit" into one list.

1. Log in to OpenEMR as each rendering clinician with MFA. Calendar shows their availability and the visit categories.
2. Open a TEST patient assigned to each facility archetype in use. Confirm the encounter picks up the correct POS.
3. In OpenEMR: new encounter → Fee Sheet → add a home E/M code → fee auto-populates at the charge-master rate, not the Medicare rate.
4. In the app, as a clinician: open TEST PatientOne's chart → follow-up note → diagnosis (T1 carry-forward shows the OpenEMR problem with its ICD-10 code) → prescription → order → service code linked to the diagnosis → sign and close. Attestation shows name, credential, NPI, timestamp.
5. In OpenEMR: the charge from step 4 is visible in Fees → Billing Manager with the right code, provider, and diagnosis pointer, and the encounter's structured note carries the `[GFC ATTESTATION]` block.
6. Attempt to bill a psych code as an A1 clinician. Blocked (by the app once 4.5 lands, by back-office review until then).
7. Add an AWW plus a problem E/M on the same day with modifier 25. Both appear on the claim.
8. Billing Manager → the encounter appears as a billable claim with Availity as its X12 partner. Generate the 837. Do not transmit until the first real claim is approved.
9. Mark a TEST patient QMB. Attempt to generate a statement. Blocked.
10. Post a sample 835 (Availity provides test files) through Fees → Posting Payments. Patient responsibility appears on the encounter. Trace number recorded on the deposit sheet.
11. Token request succeeds against the v3 client; `GET /fhir/Patient` returns the test patient; the app's sync indicator shows the 8.4 scope count; a non-clinical app user gets 403 on every clinical route.
12. Administration → System → Logs: every action above appears, attributed to the right OpenEMR user (the app's actions to `gfc-app-api`, with the acting clinician visible in the app's activity log).
13. RDS shows an encrypted database with backups; yesterday's EBS snapshot exists; the "post-8.4 clean baseline" snapshot exists; the \$80 budget alarm is armed.
14. The 6B patch diff is in the repo and the derived image tag (or override) is recorded in the compose file.

B · Values worksheet

Keep with the password manager. Names of secrets, never contents.

VALUE	SOURCE
RDS endpoint	Phase 2
Elastic IP	Phase 3

VALUE	SOURCE
EMR URL	https://emr.godwinsfamilycarellc.com
DB-MASTER / DB-OPENEMR / OE-ADMIN	Password manager; rotate before Session 5 (C-9)
OAuth2 client_id and client_secret (v3, 8.4 scopes)	AWS Secrets Manager, gfc/openemr/oauth-client
gfc-app-api username and password	Same secret
Key pair gfc-emr-key.pem	Break-glass only; currently in the Desktop GFCLLC folder, move it out of any synced folder
OpenEMR numeric ids: F1 facility, Hickory Log facility, F7 telehealth, each provider, each calendar category	Phases 8.3, 8.4, 10.4 → app env and user records
GFC EIN	—
GFC Type 2 NPI, home care	1710678339 (taxonomy correction pending)
GFC Type 2 NPI, clinical	Pending
Bethel NPI / Thanmayie NPI	1902310568 / 1962038307
Availity Customer ID; ISA/GS receiver values	Availity registration; EDI Connection Guide
PECOS login; NPPES login; CAQH ProView logins	Password manager
Clinical bank account; CMS-588 confirmation	Bank; PECOS
Capability statement file; Swagger URL	docs/openemr/capability-8.4.json ; /swagger
6B patch record	docs/openemr-patches/ (files, diff, date, version, image tag)

C · Open items that block a real claim

None of these stop Tuesday's test visit on test data. All of them stop a real claim going out.

GROUP	ITEM
Identity and enrollment	Resolve the Acworth vs Paces Ferry address conflict, then align NPPES, PECOS, 855B, IRS, OpenEMR · add 363LP0808X to NPI 1962038307 in NPPES · correct the taxonomy on 1710678339 and obtain the clinical Type 2 NPI · file the CMS-855B under GFC LLC and reassignments through PECOS · PECOS status for the group and each clinician · outside-employment exclusivity check for any clinician with another employer
Facilities	Hickory Log DCH license type, number, bed count → POS → facility record
Compliance	OIG LEIE and SAM.gov screening for the full roster · BAA with Availity, each facility, the telehealth platform, the care-management platform · write-off/adjustment/refund policy · ABN process · credential tracker populated · MFA rollout complete · tested restore performed · secrets rotated · any GFC document still referencing a standalone CMS-855R

GROUP	ITEM
Clinical operations	Delegating physician identified with scope covering every archetype and capacity for the chart-review load · Schedule II referral pathway documented · face sheets, MBLs, payer order, POA billing contacts, QMB and MA flags, PCP information per patient · AWV history pull from Palmetto eServices · MA roster split, medical and behavioral
Billing consultant (Dianne)	Confirm the CPT set GFC bills (including whether 99343's omission stands and which G/99 codes to add) · ICD-10 favorites for the panel from Palmetto LCDs · documentation thresholds per E/M level (blocks the app's T3 E/M suggestion) · whether an FNP in Georgia bills independently under her own NPI · refund/adjustment thresholds · whether an accounting system should be fed
App	Set <code>billing_npi_used</code> in Coding queue → Billing settings · set each clinician's NPI and <code>openEmrProviderId</code> · re-document pre-4.4 test encounters · deploy the v3 client credentials · run Session 4.5 then 4.3 · Session 5 (per-user OpenEMR auth, <code>audit_log</code> , PHI gate) before any real patient
Psych go-live	Both certifications verified for the A2 clinician · 855I and reassignment approved · GCMB protocol agreement with psych scope explicit · DEA and PDMP active · psych codes and note templates enabled · telehealth platform, BAA, consent workflow if telehealth is in scope · written Palmetto confirmation on whether a PMHNP satisfies the CoCM psychiatric consultant definition
Care management go-live	Platform selected and BAA executed · integration lanes verified both directions · time-log export tested and retained · consent language in the initiating-visit template · per-patient check that no other practitioner bills CCM · 24/7 arrangement staffed · program-level time separation confirmed · monthly reconciliation report · cost-sharing disclosure script
Structural	Separate books, bank accounts, and insurance by service line · CM&F COI scope verified for behavioral health rendered by an NP · written trigger for revisiting the entity split · Georgia healthcare attorney call on professional entity ownership · LCSW vs LMSW determination for any social worker on staff

D · Deferred, deliberately

ITEM	WHY IT WAITS	TRIGGER
ERA auto-posting	Manual posting first teaches payer denial behavior	After several clean months
Availity SFTP and cron	Manual upload is five minutes per batch; it moves files, it is not eligibility	Volume
Automated eligibility (B1) and Stripe collection with ledger write-back (B2/B5)	Needs Session 5 go-live and the billing architecture spec	B-series
System-to-system OAuth2 client (<code>client_credentials</code> , JWKS)	No background job exists yet	First background job (4.2b appointment-confirmation trigger), behind the HIPAA gate

ITEM	WHY IT WAITS	TRIGGER
Per-user OpenEMR auth (authorization_code + refresh)	Session 5 scope; the attribution interim covers test use	Before any real patient encounter
Quest HL7 lab interface; e-prescribing (Surescripts) and EPCS	Multi-month efforts; transmission stays manual; owner decision	Own track, post go-live
Advanced Primary Care Management (APCM)	Newer alternative to CCM with no time threshold	After the first CCM quarter
RPM codes and devices	Track C	Post-revision
A7 billing account	ACL group configured and empty	Biller hired
Multi-AZ database, larger instance	Sub-10-minute upgrades that lose no data	Volume
Entity split	7.2	Written trigger, to be set
Upstreaming the 6B patch	Nice to have; removes the monthly re-apply	When the patch is stable for a month

E · Engineer review brief

When you bring in an engineer to review the build, hand them this guide, read-only AWS console access (an IAM user with ReadOnlyAccess, never your own login), the capability statement file and Swagger URL, an OpenEMR Administrators login created for them with MFA and deleted when the review ends, the values worksheet without contents, the 6B patch record, and the VII.4 matrix with the Verified column filled in. A competent reviewer needs two to four hours. Ask them to sign off on five questions, in writing:

1. **Boundary.** Every PHI-bearing component (EC2, RDS, snapshots) encrypted, inside the BAA'd account, the database unreachable from the internet, security groups no wider than Phase 1.
2. **Recoverability.** RDS backups and EBS snapshots exist, and a test restore of each produces a working system. Ask them to perform the drill, not inspect settings.
3. **Access.** OpenEMR accounts match the 8.5 table, MFA on all, no shared logins, audit logging capturing patient-record views with correct attribution, the app's activity log covering what OpenEMR attributes to `gfc-app-api`.
4. **Integration contract.** Every VII.4 row verified against the live 8.4 server; the OAuth2 client's scopes are the minimum the app needs; the 6B patch is narrow, recorded, and survives a pull.
5. **Operations.** The monthly routine (C-10) is sound for this stack, and anything about the single-server design they would escalate before first real PHI rather than after, as a written list.

Scope discipline for the reviewer

They are auditing the build against this guide and flagging risk, not redesigning the architecture. The single-server design at launch volume is a documented decision. Improvements land in Appendix D with a trigger unless they are pre-PHI safety issues.

F · Sources

GFC documents: Deploy & Setup Guide v2.3 (July 22, 2026); Billing & Configuration Guide v3.0 (September 3, 2026); Release Runway (September 5, 2026); GFC_App_Build_v2 §1, §4, §6, §13–15; GFC_Clinical_Completeness_Spec_v1 rev 1.1; GFC_Billing_Segment_App_Integration_Spec; OPENEMR_SERVER_DEFECTS_2026-08 (with the 2026-09-04 preflight); repo `config.js`, `openemr.js`, `.env.example`, CLAUDE.md at 2026-09-05. Regulatory: 42 CFR 424.502, 424.520, 424.80, 410.73; CMS-855B/855I consolidation bulletin; CMS reassignment and NPI subpart guidance; CMS Place of Service code set; Georgia nurse protocol agreement rules effective 05/25/2026; O.C.G.A. § 43-34-25; GCMB; CMS 2026 Part B premiums and deductibles; CMS QMB program; CMS AWV; OIG LEIE; NASW 2026 PFS highlights; APA CoCM/BHI FAQs; NPPES. Platform: OpenEMR Docker image documentation (env vars, external MySQL, Let's Encrypt via DOMAIN/EMAIL); OpenEMR API authentication docs; OpenEMR 8.4 source (route table, BillingUtilities, ProcedureService, VitalsCalculatedService, DocumentService); Availity EDI Connection Services guide; AWS HIPAA-eligible services reference. Menu labels vary across OpenEMR releases; where a path does not match, use the search box in the left menu and correct the path here.

Godwins Family Care LLC · Internal reference · v4.0 · This guide configures software and records operating procedure. It is not legal, tax, or coding advice; the billing consultant and healthcare counsel decide the items marked for them in Appendix C.